

11. 9:00 AM CASE NUMBER: C25-03103
CASE NAME: CYLUS JASIAH MARTIN-HAREN, VS. LINDA CHU, M.D.
*HEARING ON MOTION IN RE: SEAL DECLARATIONS IN SUPPORT OF MOTION TO COMPEL
ARBITRATION THAT CONTAIN CONFIDENTIAL INFORMATION
FILED BY: CHU, M.D., LINDA
TENTATIVE RULING:

This Motion is unopposed. Good cause shown, it is **granted**.

12. 9:00 AM CASE NUMBER: C25-03103
CASE NAME: CYLUS JASIAH MARTIN-HAREN, VS. LINDA CHU, M.D.
*MOTION/PETITION TO COMPEL ARBITRATION
FILED BY: CHU, M.D., LINDA
TENTATIVE RULING:

Defendants Kaiser Foundation Hospitals, Kaiser Foundation Health Plan, Inc., The Permanente Medical Group, Inc. and Linda Chu, M.D.'s motion to compel arbitration is **denied**.

Plaintiff Cylus, a minor, is suing through her guardian ad litem for medical injuries caused soon after his birth on November 10, 2017. Plaintiff's guardian ad litem is his mother Gia. Plaintiff has brought one claim for medical negligence against all the defendants. Gia was also a minor when she gave birth to Cylus.

The Kaiser Defendants and Dr. Chu have brought a motion to compel arbitration. The arbitration clause at issue here comes from an arbitration clause in the Medi-Cal agreement under which Gia and later Cylus were enrolled to receive medical care from Kaiser.

Gia was enrolled in Kaiser starting from August 1, 2012 to January 1, 2024. (Deniz dec. ¶3 and ex. C.) Cylus was enrolled in Kaiser automatically in the month he was born and the month after birth. (Deniz Dec. ¶ 4.) Cylus was then enrolled in Kaiser from March 1, 2018 to July 1, 2025. (Deniz dec. ¶ 4 and ex. D.)

Records from CCHP show two phone calls regarding Plaintiff and/or his mother's healthcare coverage. (Pierrou dec. ex. N.) The Court sustained Plaintiff's objections to these records and therefore, the Court will not consider them further.

The 2017-2018 Medi-Cal Member handbook includes an arbitration clause. (Velarde dec. ex. A at p. 92-95.) The arbitration clause requires members to use binding arbitration for malpractice claims. (Id. at 92.) The 2012-2023 Medi-Cal Member handbook includes a similar arbitration clause. (Velarde dec. ex. B. at 83-84.)

The arbitration clause at issue here covers the disputes alleged in the complaint for

malpractice. The first question is whether Plaintiff can be bound by this arbitration agreement. There is no showing that Plaintiff, Gia or Melissa signed an arbitration clause purporting to agree to arbitration for Gia and/or Plaintiff.

Code of Civil Procedure section 1281.2 allows a party to compel arbitration based upon a written arbitration agreement. That section does not require that the arbitration agreement be signed.

In *Madden v. Kaiser Found. Hosps.* (1976) 17 Cal.3d 699, 703-704, 708 the California Supreme Court held that an employer, acting as agent of its employees, has implied authority to agree to binding arbitration of malpractice claims arising under a health services plan it negotiates as part of an employee benefit package. The Court held that “an agent or other fiduciary who contracts for medical treatment on behalf of his beneficiary retains the authority to enter into an agreement providing for arbitration of claims for medical malpractice.” (*Id.* at 709.)

Actual knowledge of the arbitration provision is not required. (*Id.* at 709 fn. 11.) However, the cases in this area require some evidence showing that the arbitration clause was given to the member, usually done by mailing a copy of the evidence of coverage and related documents.

Kaiser contracts with the California Department of Health Care Services (DHCS) regarding its Medi-Cal coverage. Kaiser also previously contracted with Contra Costa Health Plan Member Services (CCHP) regarding Medi-Cal. (Velarde dec. ¶1.) CCHP contracts with DHCS to arrange for health care services for eligible Medi-Cal beneficiaries in Contra Costa County. Until 2024, CCHP contracted with Kaiser, as a Medi-Cal managed care plan, to provide and arrange for health care services. (Velarde dec. 2.) Kaiser member handbooks are required to follow certain rules and go through regulatory approval by DHCS or CCHP before they can be distributed to members. (Velarde dec. ¶¶3-4.)

DHCS and/or CCHP exclusively managed and carried out the process by which Gia and Cylus were enrolled as Medi-Cal members. Kaiser does not enroll Medi-Cal members directly. Kaiser must accept members that are eligible as Medi-Cal beneficiaries and meet certain criteria. Once a person completes the Medi-Cal enrollment process with DHCS (or its’ contractors) then Kaiser must accept them as members. (Deniz dec. ¶15.)

Kaiser’s evidence also shows that they mailed a copy of the 2016 annual medi-cal member notice and portions of the 2016-2017 CCHP member handbook to Gia’s household in 2016, which included information on the binding arbitration clause. (Guha dec. ¶13.) There is also evidence that Gia’s household was mailed other documents including the arbitration clause after the malpractice at issue here, but it is not clear how those would bind Plaintiff.

The Court finds that Kaiser’s evidence is sufficient to show that Kaiser contracts with DHCS to provide health care services for Medi-Cal recipients and that DHCS or CCHP approved the member handbook that contained the arbitration clause. Following the reasoning in *Madden*, DHCS and/or

CCHP were acting as the agent for members receiving health care services under Medi-Cal and that those government agencies could agree to an arbitration clause on behalf of the Medi-Cal members.

The Court finds then that Kaiser has met its initial burden of showing that a valid arbitration agreement exists. The burden now shifts to Plaintiff to show a reason why the arbitration agreement is not valid.

Plaintiff argues that neither Gia nor Melissa remember getting a copy of the member handbook. (Melissa dec. ¶6; Gia dec. ¶7.) These general statements are insufficient to show that Kaiser did not mail out the member handbook.

Health & Safety Code section 1363.1

Plaintiff argues that Kaiser did not comply with Health and Safety Code section 1363.1 and as such is prohibited from enforcing the arbitration clause at issue here. Section 1363.1 requires that health care plans with binding arbitration must include a disclosure of binding arbitration and the waiver of a right to a jury trial. The required disclosure must be immediately before the signature line provided for the individual enrolling in the health care service plan. (Health & Safety section 1363.1.) State regulations require that that evidence of coverage, disclosure forms and marketing brochures comply with section 1363.1. (Cal. Code Regs., tit. 22, § 53881.)

After *Madden*, Health and Safety Code section 1363.1 was enacted to ensure that members had notice of arbitration and knowing waived the right to a jury trial. “The purpose of section 1363.1 is to disclose the requirement to arbitrate *and* to ensure a knowing waiver of the right to a jury trial. The signature of the ‘individual enrolling in the health service plan’ is required in order to ensure a knowing waiver of his or her rights.” (*Malek v. Blue Cross of California* (2004) 121 Cal.App.4th 44, 72.)

“Rather than abrogating the *Madden* decision by requiring an individual waiver of right to jury trial by a person enrolling in a health care service plan requiring binding arbitration, the Legislature extended to enrollees the more limited protection of disclosure.” (*Viola v. Dep’t of Managed Health Care* (2005) 133 Cal.App.4th 299, 312.) An entity is “empowered under *Madden* to negotiate a waiver of jury trial on behalf of” enrollees only “so long as the plan complies with the disclosure requirements of section 1363.1.” (*Id.* at pp. 312, 314.)

It is undisputed that when Gia and then Cylus were enrolled in Kaiser no one signed as required section 1363.1 disclosure and there is no evidence that Melissa, Gia or Cylus was given the disclosure when enrolling Gia and/or Cylus. Defendants argue, however, that section 1363.1 is preempted by two federal laws.

Defendants argue that the FAA preempts section 1363.1 because it treats arbitration clauses differently from other contracts. However, the McCarran-Ferguson Act, 15 USCS § 1012, prevents federal laws that do not specifically relate to the business of insurance from invalidating a state law

enacted for the purpose of regulating the business of insurance. Most California courts have rejected this preemption argument and the Court finds them persuasive. (*Imbler v. PacifiCare of Cal., Inc.* (2002) 103 Cal. App. 4th 567, 576–77; *Pagarigan v. Superior Court* (2002) 102 Cal.App.4th 1121, 1134; *Smith v. PacifiCare Behavioral Health Plan of Cal., Inc.* (2001) 93 Cal.App.4th 139, 143.) Therefore, the Court finds that section 1363.1 is not preempted by the FAA.

Defendants also argue that federal Medicaid law preempts section 1363.1. Preemption includes when “[a] state law actually conflicts with federal law ‘where it is impossible for a private party to comply with both state and federal requirements [citation], or where state law “stands as an obstacle to the accomplishment and execution of the full purposes and objectives of Congress.” ’ [Citation.]” (*Olszewski v. Scripps Health* (2003) 30 Cal.4th 798, 815.)

“In construing [a state] statute, “‘our fundamental task is “to ascertain the intent of the lawmakers so as to effectuate the purpose of the statute.”’” [Citation.] ‘We begin by examining the statute’s words, giving them a plain and commonsense meaning. [Citation.] We do not, however, consider the statutory language “in isolation.” ... [W]e construe the words in question “in context, keeping in mind the nature and obvious purpose of the statute”’ [Citation.] We also ‘construe every statute with reference to the whole system of law of which it is a part, so that all may be harmonized and anomalies avoided.’ [Citations.] Further, in the preemption context, we adhere to ‘the rule that courts should, if reasonably possible, construe a statute “in a manner that avoids *any* doubt about its [constitutional] validity.”’ [Citations.]” (*Hohenshelt v. Superior Court* (2025) 18 Cal.5th 310, 330.)

Under the Affordable Care Act, agencies must accept applications for health care via the internet, telephone, mail, in person or other commonly available electronic means. (42 C.F.R. § 435.907; see also, Welf. & Inst. Code, § 15926 [listing similar requirements].) Applications must be signed under penalty of perjury, however, “[e]lectronic, including telephonically recorded, signatures and handwritten signatures transmitted via any other electronic transmission must be accepted.” (42 C.F.R. § 435.907 (f).)

California also allows for electronic signatures. DHCS issued a letter in 2019 that states county departments must allow telephonic signatures when enrolling Medi-Cal members. (Cal. Dept. of Health Care Services, All County Welfare Directors Letter No. 19-17, Telephonic or Electronic Signature Capability (June 21, 2019), currently found at <https://www.dhcs.ca.gov/wp-content/uploads/2025/10/c19-17.pdf>.) DHCS pointed to two portions of the Welfare and Institutions Code that specifically permitted telephonic signatures (sections 14005.37, 14014.5) and the rules in the Code of Federal Regulations allowing telephonically recorded signatures.

The ACA also required that there be one streamlined form used to apply state subsidized healthcare programs, which would include Medi-Cal. (42 U.S.C. § 18083.) Eligible individuals are enrolled automatically if they fail to select a plan. (42 U.S.C. § 1396u-2(a)(4)(D); 22 C.C.R. section

53883(a).)

In 2002, *Pagarigan* held that section 1363.1 as not preempted by Medicare. (*Pagarigan, supra*, 102 Cal.App.4th at 1149.) *Pagarigan* was looking at an older version of the law in the context of Medicare, not Medi-Cal. In dicta, the court also noted that the 2000 amendments applying to Medicare marketing materials would preempt state law on Medicare marketing materials in the future. (*Id.* at 1150.) Here, however, Defendants are not arguing that the rules for Medicare marketing (42 U.S.C. § 1395w-21 (h)(5)) are the basis of preemption when someone enrolls in Medi-Cal.

There is no other published state court opinion addressing this preemption issue. There are a few federal district court opinions that have considered this issue and found Medicaid preempts section 1363.1. (*Wills v. Kaiser Found. Hosps.* (N.D.Cal. Nov. 6, 2025, No. 25-cv-01948-LB) 2025 LX 478387, at *10; *Hunter v. Kaiser Found. Health Plan, Inc.* (N.D.Cal. 2020) 434 F. Supp. 3d 764, 772; *Tidwell v. United States HHS* (N.D.Cal. Mar. 10, 2026, No. 25-cv-04777-ASK) 2026 LX 123792, at *8; see also, *Winninger v. Scott* (N.D.Cal. July 5, 2022, No. 21-cv-04689-HSG) 2022 LX 34143, at *4 [considering 10 C.C.R. 6470.]) These federal cases are not binding, and the Court is not convinced by the analysis provided in these cases that section 1363.1 is preempted.

The purpose of section 1363.1 is to disclose the requirement to arbitrate and to ensure a knowing waiver of the right to a jury trial. Its language suggests that the disclosure would be on a written form, however, it does not contemplate enrollment for healthcare services over the phone. Current state and federal law allows for telephonically recorded signatures as part of enrollment (and redeterminations of eligibility) in Medi-Cal. Section 1363.1 can be construed as allowing a verbal disclosure and a recorded telephonic signature when a person enrolls in Medi-Cal over the phone. This interpretation is consistent with the purpose of the statute and this construction avoids doubts about preemption.

The Court finds that section 1363.1 is not preempted in this situation. Federal Medicaid law requires one streamlined application and requires that states allow for enrollment over the telephone. But these rules do not conflict with compliance under section 1363.1 when it is interpreted to allow a verbal disclosure and an electronic recording of the signature by the enrollee.

Plaintiff argues that California regulations require specific disclosure language regarding Kaiser and arbitration when a party enrolls through Covered California. (10 C.C.R. § 6470 (f)(2)(B).) However, this regulation is part of the “California Health Benefit Exchange”, also known as Covered California, and it is not clear that this regulation applies to requests for enrollment outside of Covered California. (10 C.C.R. § 6410.)

Other Defenses to Enforcement

Plaintiff also argues that he is a minor and permitted to disaffirm contracts and he does so with the arbitration clause. (Fam. Code §§ 6700, 6710 (a minor may disaffirm his contracts).) Although

the Court finds that section 1363.1 was not complied with in this case, it has also provided its analysis on this issue in order to include a full discussion of the issues raised in this motion.

Plaintiff notes that a minor cannot disaffirm a contract for medical services that includes an arbitration clause, but only where the contract is signed by the minor's parent or legal guardian. (Code Civ. Proc., § 1295 (d).) Here, there is no signature by Gia or Melissa binding Plaintiff to arbitration.

In *Doyle v. Giuliucci* (1965) 62 Cal.2d 606, 610 the court held that the contract between a minor plaintiff's father and a medical defendant provided for arbitration of her claim and that she was bound by that agreement. (*Id.* at 610.) The court explained that the contract for medical services was between the father and the provider and that the minor was a third-party beneficiary of the contract.

The Court finds that Plaintiff has not shown he has a right to disaffirm the contract for medical care because he was a third-party beneficiary of the agreement between Kaiser and DHCS to provide medical care to Plaintiff.

Defendant Chu

Defendants argue that Chu was a Kaiser doctor at the time of the alleged malpractice, however, the evidence on that point was properly objected to by Plaintiff. It seems likely that Kaiser will be able to provide additional evidence that Chu was a Kaiser doctor. Further, it is not clear that the parties really dispute that Chu should remain in state court if the parties are required to arbitrate their dispute with Kaiser. Since the Court finds that section 1363.1 was not complied with in this case, there is no need to resolve this issue regarding whether Chu was a Kaiser doctor at the relevant time.

Evidentiary Rulings

The Court rules on Plaintiff's objections as follows:

1. Sustained.
2. Sustained.
3. Sustained.
4. Sustained. Printouts from 2026 are not relevant to the time periods at issue in this motion.
5. Sustained. A printout from 2023 is not relevant to the time periods at issue in this motion.
6. Sustained.
7. Sustained as to "always". Otherwise overruled. The Court interprets this statement to apply from 2010 to the present.
8. Overruled.
9. Overruled.
10. Overruled.
11. Overruled.
12. Overruled.

- 13. Overruled.
- 14. Overruled.
- 15. Overruled.
- 16. Overruled.
- 17. Overruled.
- 18. Overruled.
- 19. Overruled.
- 20. Sustained.

13. 9:00 AM CASE NUMBER: C26-00456
CASE NAME: SEBRION MCHENRY-STEWART VS. JESSE GILLESPIE
HEARING IN RE: MOTION TO CONSOLIDATE CASES
FILED BY:
TENTATIVE RULING:

The Court denies this Motion without prejudice for lack of proof of service on defendants in both matters.

14. 9:00 AM CASE NUMBER: C26-00702
CASE NAME: ABDUL KHAMOSH VS. ZAKIA KHAMOSH WAKILY
***HEARING ON MOTION IN RE: STRIKE PURSUANT TO CCP 425.16**
FILED BY: KHAMOSH WAKILY, ZAKIA
TENTATIVE RULING:

Defendants' special motion to strike pursuant to Code of Civil Procedure section 425.16 is **denied**.

Background

Plaintiff Abdul Salam Khamosh alleges that he is a prominent businessman in the Afghan-American community who operates multiple gas station businesses. (Plaintiff's February 27, 2026 Complaint ["Compl."] ¶ 7.) Between 2008 and 2013, Plaintiff personally financed, advanced, and managed his late father Dr. Mohammed Aslam Khamosh's legal claims against the United States government regarding ancestral land in Kapisa Province, Afghanistan, incurring hundreds of thousands of dollars in out-of-pocket expenses and over \$1 million in lost income. (Compl. ¶¶ 17-23.) In recognition of these personal and financial sacrifices, his father executed a California Statutory Will on January 25, 2013, devising his entire estate to Plaintiff. (Compl. ¶ 24.) Following the father's death in January 2016, the federal claim settled for \$195,000, which Plaintiff alleges constituted the sole asset of the estate. (Compl. ¶¶ 25-26.) Plaintiff petitioned for probate in Contra Costa County Superior Court under Case No. P16-00362. (Compl. ¶ 28.) Defendant Zakia Khamosh Wakily ("Wakily") filed an objection and will contest strictly limited to five grounds: improper statutory execution/attestation, lack of testamentary intent, revocation by a purported living trust, mistake of fact regarding the purpose of the document, and undue influence. (Compl. ¶ 29.) The 2016 will contest contained no allegations of criminal forgery, fraud, theft of estate assets, or overseas bank embezzlement. (*Id.* ¶ 29; Defs.' RJN, Exh. 2.) In response to the contest, Defendants Mohammed Omar Khamosh ("Omar") and Najia Khair ("Khair") submitted sworn declarations under penalty of perjury attesting that the will reflected their father's intent, that the estate possessed no assets other than the \$195,000 federal settlement, and that Wakily's accusations against Plaintiff were "vicious lies" that were "totally untrue." (Compl. ¶¶